CHIEF COMPLAINT:

Severe pruritic, dry, erythematous skin rash with excoriations over the flexural surfaces of both upper and lower extremities for six weeks.

HISTORY OF PRESENT ILLNESS:

The patient is a 24-year-old female with a childhood history of allergic rhinitis and atopic dermatitis presenting with a recalcitrant flare of eczema. She reports unbearable itching, particularly at night, leading to significant sleep fragmentation. Over-the-counter hydrocortisone cream and petroleum-based moisturizers have provided minimal relief. Over the past four days, she has noticed honey-colored crusting and increased warmth in her arm creases.

PHYSICAL EXAMINATION:

Dermatologic exam reveals confluent erythematous, xerotic plaques with marked lichenification, fissuring, and honey-colored crusting overlying the bilateral antecubital and popliteal fossae. Multiple linear scratch marks (excoriations) noted across both forearms. Auspitz sign negative. Scalp and nails are uninvolved. No mucosal lesions.

ASSESSMENT AND PLAN:

1. Severe chronic atopic dermatitis flare with secondary impetiginization.

2. Prescribe topical triamcinolone acetonide 0.1% ointment to be applied twice daily to affected flexural areas.

3. Prescribe oral cephalexin 500 mg TID for 7 days for superficial bacterial colonization.

4. Prescribe oral hydroxyzine 25 mg at bedtime for nocturnal pruritus.

5. Emphasize liberal application of ceramide-based emollients immediately following lukewarm baths.